[redacted]

Fax

[redacted]

To: [redacted] Fax: [redacted]

From: [redacted] Date: [redacted]

Re: Recup Auth Request Pages: 23

Cc: [redacted]

☐ Urgent X For Review ☐ Please Comment ☐ Please Reply ☐ Please Recycle

Comments

Name [redacted]

CIN [redacted]

Attachments:

STPH Referral, Hospital Clinicals

Contact email [redacted]

IMPORTANT: This facsimile transmission contains confidential information, some or all of which may be protected health information as defined by the federal Health Insurance Portability & Accountability Act (HIPAA) Privacy Rule. This transmission is intended for the exclusive use of the individual or entity to whom it is addressed and [redacted] contain information that is proprietary, privileged, confidential and/or exempt from disclosure under applicable law.

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L.A. Care HEALTH PLAN

Short Term Post-Hospitalization Housing
Service Authorization Request Form

Please fax completed document t[redacted]

[redacted]

Short-Term Post-Hospitalization Housing (STPHH) provides Members who do not have a residence and who have high medical or behavioral health needs with the opportunity to continue their medical/psychiatric/substance use disorder recovery.

STPHH is one of the Room and Board services, along with Transitional Rent and Recuperative Care, and is subject to a global cap. Under this global cap, Members [redacted] **not** receive more than six months (182 days) of Room and Board services per rolling 12-month period.

This form is intended to be used only for L.A. Care Members and should not be used for Members enrolled with Anthem and Blue Shield.

An asterisk (*) indicates a required field.

Attestations

Has the Member utilized Recuperative Care, STPHH and/or Transitional Rent through L.A. Care or another L.A. County Medi-Cal Managed Care Plan (MCP) in the last 12 months?* ☑ Yes ☐ No ☐ Unknown

If yes, which MCP provided services? LA Care

How many total combined days has the Member utilized Recuperative Care, STPHH and/or Transitional Rent services?* 90 days

Please use this section to provide additional comments about the Member's utilization history under the global cap.

Member admitted t[redacted]

☑ Check here if the Member has been informed there is a coverage limit of six months of Recuperative Care, STPHH and/or Transitional Rent services per Member within a rolling 12-month period.*

☑ Check here to attest that the Member meets the level of care criteria for STPHH and does not require a higher level of care that cannot be met by STPHH placement.*

☑ Check here if you have obtained "Member consent" to enroll (Opt-in) into L.A. Care Health Plan's STPHH Program and you will be able to present documentation substantiating this Member's consent upon any prospective future audit.*

Eligibility

Please check the criteria the Member meets for STPHH.

☑ Member is enrolled in L.A. Care Medi-Cal (MCLA) or L.A. Care Medicare Plus (HMO D-SNP)* **AND**

☑ Member is exiting an institution* **AND**

☑ Member is experiencing homelessness or is at risk of homelessness* **AND**

Member meets one of the following criteria (select all that apply)*

- ☑ Member is receiving Enhanced Care Management (ECM)
- ☑ Member has one or more chronic conditions
- ☑ Member has serious mental illness **OR**
- ☐ Member is at risk of institutionalization or requiring residential services such as a result of having a substance use disorder **AND**
- ☐ Member has ongoing physical or behavioral health needs as determined by a qualified health professional that would otherwise require continued institutional care if not for receipt of STPHH.

L.A. Care HEALTH PLAN

Short Term Post-Hospitalization Housing
Service Authorization Request Form

Please fax completed document t[redacted]

Member Information

Member's First Name:* [redacted]
Member's Last Name:* [redacted]
Member's Medi-Cal Number: [redacted]
Member's Date of Birth: [redacted]
Member's Phone Number: [redacted]
Member's Email Address:* none
Member's Contact Preference:* ☑ Phone ☐ Email
Member's Gender:* ☐ Female ☑ Male ☐ Transgender Female ☐ Transgender Male ☐ Non-Binary ☐ Other _____
Is the Member receiving any of the following services?* (select all that apply)
☑ Enhanced Care Management (ECM). If yes, which Provider? [redacted]
☐ Housing Navigation. If yes, which Provider? _____
☐ Tenancy Sustaining Services. If yes, which services? _____
☐ Unknown

Member Current Location

☐ Inpatient hospital (acute, psychiatric, chemical dependency or recovery hospital)
☐ Residential Substance Use Disorder or recovery facility ☐ Residential mental health treatment facility
☐ Correctional facility ☐ Nursing facility ☑ Recuperative Care
Name of the institution where the Member is located:* [redacted]
Date of Admission:* [redacted] Requested STPHH Authorization Start Date:* [redacted]
☑ Check here if facility progress notes are attached (required if exiting Recuperative Care ONLY)

Referral Source Information

Date of Referral:* [redacted]
L.A. Care Internal referring department* (select one): ☐ BH ☑ CM ☐ MLTSS ☐ SS ☐ Other: _____
External referral by (select one) :* ☐ Clinic ☐ ECM ☐ Hospital ☐ SNF ☐ PCP ☐ PPG ☑ Recup ☐ Field Medicine
Other _____
Referring Individual's Name:* [redacted]
Referring Individual's Organization Name:* [redacted]
Referrer Phone Number:* [redacted]
Referrer Fax Number:* [redacted]
Referrer Email Address:* [redacted]
Alternative Contact Name: [redacted]
Alternative Contact Phone Number:* [redacted]
Alternative Contact Email Address:* [redacted]
Referring Individual's Relationship to the Member:* [redacted] ☑ Social Services Provider ☐ Member/Family
☐ Other: _____

L.A. Care HEALTH PLAN

Short Term Post-Hospitalization Housing
Service Authorization Request Form

Please fax completed document t[redacted]

[redacted]

Health Information

Height: Unk Weight: Unk Allergies: NKDA

General Medical Diagnosis/Problems: Resp Failure, AFib, Asthma, Hypertension

Mental Health/Substance Use Diagnosis/Problems: anxiety, depression

1. Can the Member Self-Represent? ☑ Yes ☐ No
2. Does the Member have impaired cognition? ☐ Yes ☑ No
3. Is the Member independent w/ ADLs? ☑ Yes ☐ No

   If NO, please explain: _____

4. Can the Member ambulate stairs independently? ☑ Yes ☐ No
5. Is the Member dependent on the following DME? ☐ Walker ☐ Cane ☐ Crutches ☐ Wheelchair ☐ N/A
6. Will home health continue to follow the Member at STPHH? ☐ Yes ☑ No ☐ N/A

   If YES, name of Home Health provider _____ Phone Number _____

7. Does the Member require isolation? ☐ Yes ☑ No

   If YES, type of isolation _____

8. Does the Member use any substances? (Check all that apply)
   - ☐ Alcohol Use
   - ☐ Opioid Use (Heroin, Fentanyl)
   - ☐ Stimulants Use (Cocaine, Methamphetamines)
   - ☐ Other _____

The following documents are frequently requested by STPHH facilities. Please include with submission if available:

1. Face Sheet
2. History and Physical
3. Medication List
4. Wound Care Notes
5. Psych Notes (if applicable)
6. Assessment of ADLs/IADLs

Referring entities can work directly with an in-network L.A. Care contracted STPHH facility. When working directly with a STPHH facility you do not need to submit a duplicative referral directly to L.A. Care. You do not need to send L.A. Care any additional clinical documentation, unless the STPHH facility is unable to accept your referral, and you chose to have L.A. Care assist with your request. For a list of L.A. Care STPHH Providers, please click here.

For any questions, please emai[redacted]

[redacted]

[redacted]

[redacted]

[redacted]

[redacted]

Age/Sex: 55/M

Room/Bed: [redacted]

[redacted]

Type: PT-Physical Therapy Assessment

What type of intervention is this
- Initial

Physician Order
- PT Eval

Primary Diagnosis
- Acute respiratory failure with hypoxia
  Heart failure acute on chronic with systolic dysfunction
  Paroxysmal atrial fibrillation
  History of asthma
  Hypertension
  History of valvular heart disease, status post aortic valve bioprosthetic valve replacement
  [redacted]

  This is a 55-year-old male with a history of heart disease that being congestive heart failure, status post aortic valve replacement with bioprosthetic, atrial fibrillation and hypertension who presented to the emergency room with severe shortness of breath. He denies a productive cough. He denies fever.
  In the emergency room the patient required a BiPAP for adequate oxygenation. An arterial blood gas showed a pH of [redacted] He was in need of inpatient care and these arrangements were made.
  The patient had an echocardiogram done at the beginning of this year showing an LVEF of [redacted]

Medical History
- history of heart disease that being congestive heart failure, status post aortic valve replacement with bioprosthetic, atrial fibrillation and hypertension

Surgery
- [redacted]

Equipment and Lines
- Cardiac Monitor
- IV

Precautions
- Falls

Resided In
- Homeless

Assistive Devices
- None

Bed Mobility
- Independent

Transfer
- Independent

Toileting and Bathing
- Independent

Ambulation Ability
- Independent

Primary Language
- English

Comprehension Ability

[redacted]

- No Impairment

Oral Expression Ability
- No Impairment

Level of Consciousness
- Alert

Patient Orientation
- Person
- Place
- Situation

Direction Following Ability
- Simple
- Multiple

Eye Assessment Label
- Left

Range of Motion Assessment Label
- Left
  - Range of Motion - Within Functional Limit? Yes

Range of Motion Assessment Label
- Right
  - Range of Motion - Within Functional Limit? Yes

Strength Assessment Label
- Left
  - Strength - Within Functional Limit? Yes

Strength Assessment Label
- Right
  - Strength - Within Functional Limit? Yes

Movement Assessment Label
- Left
  - Upper Extremity Normal
  - Muscle Tone Normal

Movement Assessment Label
- Right
  - Upper Extremity Normal
  - Muscle Tone Normal

Muscle Tone
- Normal
- Normal

Pain Scale
- 0

Numbness and Tingling
- No

Bed Mobility Supine to Sit
- Independent

Sit to Supine Mobility
- Independent

Transfer Sit to Stand Ability
- Independent

Transfer Stand to Sit Ability
- Independent

Transfer Assistive Device
- None

Ambulation Assistance
- Independent

Ambulation - Assistive Device
- None

Ambulation Distance
- 240 ft

Static: Balance Sitting
- Normal

Static: Balance Standing
- Normal

Assessment/Problems
- Not Appro. for Skill Ther

Treatment Plan
- No Skill Thrpy indicated
- Discharge from PT

Discharge Recommendation Comment:
- D/C per MD

PT Additional Comment:
- PT Eval Moderate. D/C by PT.

R[redacted] cleared pt. to participate in PT evaluation. Pt. received in bed and agreeable to participate with PT. Pt. reporting no pain at rest or SOB episodes. Able to perform bed mobility, transfers, and gait up to 240 ft w/o AD independently without any noted instability in gait or dyspnea. Pt. to be discharged from PT due to lack of functional impairments at time of evaluation. Pt. returned back to bed with call light in arms reach and RN notified of pt. functional mobility status and disposition.

[redacted]

Note started by: [redacted] Note started date/time: [redacted]

IM History & Physical - [redacted]

General

Date of Service

[redacted]

Time of Evaluation
18:16
Date of Admission
Date of Admission: [redacted]
Age
Age: 55
Sex: Male

Chief Complaint

Chief Complaint
Shortness of breath

History of Present Illness

HPI
This is a 55-year-old male with a history of heart disease that being congestive heart failure, status post aortic valve replacement with bioprosthetic, atrial fibrillation and hypertension who presented to the emergency room with severe shortness of breath. He denies a productive cough. He denies fever.
In the emergency room the patient required a BiPAP for adequate oxygenation. An arterial blood gas showed a pH of [redacted] He was in need of inpatient care and these arrangements were made.
The patient had an echocardiogram done at the beginning of this year showing an LVEF of [redacted]

Past Medical History

PMH
Congestive heart failure
Valvular heart disease
Hypertension
Paroxysmal Atrial fibrillation
Cerebrovascular accident
Asthma

Past Surgical History

PSH
Aortic valve replacement 2012

Medications

Meds
Norvasc 10 mg daily
Eliquis 5 mg b.i.d.
Lipitor 20 mg daily
Lasix 40 mg b.i.d.
Losartan 100 mg daily
Toprol-XL
Aldactone 25 mg daily

Allergies

Allergies:
Coded Allergies:
No Known Allergies (Verified Allergy, Unknow[redacted]

Habits

Patient Habits

Positive for [redacted]use [redacted]

Social History

Soc Hx

Patient states he is homeless

Family History

Fam Hx

Positive for hypertension

Review of Systems

General: [redacted]HPI, weight changes, recent illnesses, recent injuries, fevers, chills, diaphoresis, night sweats, other
Mouth/Throat/Neck: No: Per HPI, difficulty swallowing, mouth sores, hoarseness, neck swelling, other
Chest: No: Per HPI, lumps, pain, other, masses
Cardiac: Yes: Per HPI, hypertension, palpitations, dyspnea on exertion
Respiratory: Yes: shortness of breath
Gastrointestinal: No Per HPI, No abdominal pain, No nausea, No vomiting, No bloating, No distention, No diarrhea, No constipation, No change in stool color, No dyspepsia, No other
Genitourinal: No Per HPI, No dysuria, No urgency, No frequency, No hematuria, No nocturia, No incontinence, No hesitancy, No other
Vascular: No: Per HPI, leg pain, claudication, leg swelling, varicosities, thrombi, other
Musculoskeletal: No Per HPI, No limited ROM, No instability, No pain, No joint stiffness, No weakness, No paralysis, No joint swelling, No pain, No redness, No osteoarthritis, No rheumatoid arthritis, No gout, No other
Neurologic: No: Per HPI, Dizziness, lightheadedness, fainting, blackouts, seizures, headache, numbness, tingling, tremours, weakness, other
Hematologic: No: Per HPI, anemia, bruising, bleeding, transfusion reactions, other
Endocraine: No: Per HPI, heat or cold intolerances, excessive sweating, polyuria, polydispsia, polyphagia, other
Psychiatric: No: Per HPI, anxiety, depression, tension, memory, other

Vital Signs

Vitals

Vital Signs 48 H

Date Time	Temp	Pulse	Resp	B/P (MAP)	Pulse Ox	O2 Delivery	O2 Flow Rate	FiO2
[redacted]		76	16	146/116 (125)	95	Nasal Cannula*		44
							[redacted]	
		76	17		99	Nasal Cannula*		44
		64	19	142/110 (120)	93	Nasal Cannula*		44
		74	18	[redacted](123)	98	Nasal Cannula*		44
		56	16	130/98 (109)	99	Bipap+		35 N/A
		58	18	[redacted](107)	98	Bipap+		35 N/A
		69	18	121/96 (106)		Bipap+		35 N/A
		78	18	153/88 (109)	94	Bipap+		35 N/A
		81	14	136/124 (130)	97	Bipap+		35 N/A
								35
	37.2	74	16	151/99 (113)	98	Bipap+		35 N/A
		77	16	144/128 (136)	99	Bipap+		35 N/A
		83	14	134/99 (106)	98	Bipap+		35 N/A
		93	23		95	Bipap+		35 N/A

	90	19	155/114 (126)	96	Bipap+		35
	74	17	137/104 (115)	97	Bipap+		N/A 35
	87	18	167/124 (131)	95	Bipap+		N/A 35
		20					N/A 35
	76	19	142/108 (120)	97	Bipap+		35
							N/A 35
	74	16	124/103 (111)	97	Bipap+		35
	75	16	137/115 (124)	97	Bipap+		N/A 35
	87	25	156/114 (127)	97	Bipap+		N/A 35
	113	26	144/102 (111)	97	Bipap+		N/A 35
			144/118				N/A
	86	19	144/118 (127)	94	Bipap+		35
	80	24	138/118 (126)	99	Bipap+		N/A 35
	81	20	141/116 (125)		Bipap+		N/A 35
	80	26	141/117 (126)	98	Bipap+		N/A 35
							N/A 35
37.1	92	25	159/137 (146)		Bipap+		35
	69	17	147/107 (121)	100	Bipap+		N/A 35
	67	18	133/101 (112)	99	Bipap+		N/A 35
	75	17	137/107 (117)	94	Bipap+		N/A 35
	79	20	148/118 (129)	95	Bipap+		N/A 35
	96	20	159/123 (136)		Bipap+		N/A 35
	93	21	170/131 (144)	99	Bipap+		N/A 35
	116	17	177/161 (168)	96	Nasal Cannula*		N/A 36
	88	20	[redacted] (141)		Nasal Cannula*		36
						4.00	
	106	20	197/145 (161)	97	Nasal Cannula*		36
	112	17	172/115 (132)	98	Nasal Cannula*		36
	114	19	188/152 (164)		Nasal Cannula*		36
	113	23		99	Nasal Cannula*		36
	114	20		99	Nasal Cannula*		36
			152/114				
			152/114				
	115		152/114				
			152/114				
[redacted]	114	21	152/114 (128)	99	Nasal Cannula*		36
							100
36.9184	124	18	178/124				
	115	19	181/131 (148)	95	Bipap+		50
	113	19	177/129 (145)	95	Bipap+		N/A 50
			173/133				N/A

	113	19	167/133 (144)	95	Bipap+		50
							N/A
	113	19	163/133 (143)	95	Bipap+		50
							N/A
36.4	110	18	149/116 (127)	99	Bipap+		60
							N/A
							60
	111	19	156/115 (129)	97	Bipap+		60
							N/A
	111		160/128				
	111		161/125				
			180/153				
	115	21	160/127 (138)	97	Bipap+		60
							N/A
							50
			190/135				
		22					60
	116	36	196/158 (171)	98	Bipap+		100
							N/A
	117						
		34					100
	117						
	116	40	196/158 (171)	94	CPAP/BiPAP		

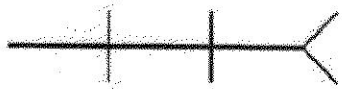
Physical Exams

General: moderate distress, other (On BiPAP)
Head: head is symmetrical
Eye: PERRL bilat and equal
Heart: S1,S2 present
Lungs: Other (Decreased breath sounds)
Abdomen: soft, non-tender, non-distended
Extremities: other (Trace edema)
Neurologic: alert, moves all ext equally

Labs

Laboratory
Laboratory Tests

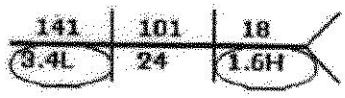




Red Blood Count [redacted] Mean Corpuscular Volume [redacted] Mean Corpuscular Hemoglobin [redacted] **Mean Corpuscular Hemoglobin Concent** [redacted] **L, Red Cell Distribution Width** [redacted] **H**, Mean Platelet Volume [redacted] Neutrophils (%) (Auto) [redacted] Lymphocytes (%) (Auto) [redacted] Monocytes (%) (Auto) [redacted] Eosinophils (%) (Auto) [redacted] Basophils (%) (Auto) [redacted] Glucose Level , Glomerular Filtration Rate Calc , Calcium Level

[redacted]





Glucose Level 238 H, Glomerular Filtration Rate Calc 55, Calcium Level [redacted]

[redacted]

Blood Gas Panel 24Hour

Blood Gas Specimen Type Blood Arterial, Blood Gas Puncture Site LEFT RADIAL, **Blood pH 7.162*L, Blood Gas PCO2**

49.3H, Blood Gas PO2 379.5*H, Blood Gas HCO3 17.3L, Blood Gas Base Excess -11.6L, Blood Gas Oxygen Saturation 100.0H, Blood Gas Oxyhemoglobin [redacted] Blood Gas Hemoglobin 18.2*H, Deoxyhemoglobin 0.0, Carboxyhemoglobin [redacted] Methemoglobin 0.2L, Blood Gas Vent Mode MASK, BIPAP, Blood [redacted] Rate 18.0, Blood Gas Inspired Oxygen 100, Blood Gas Operator Identification [redacted] Blood Gas Critic [redacted] YES, [redacted] Blood Gas Notified [redacted]

Blood Gas Specimen Type Blood Arterial, Blood Gas Puncture Site LEFT RADIAL, Blood pH 7.491H, Blood Gas PCO2 [redacted] Blood Gas PO2 80.2L, Blood Gas HCO3 20.7L, Blood Gas Base Excess -0.6, Blood Gas Oxygen Saturation 96.9, Blood Gas Oxyhemoglobin [redacted] Blood Gas Hemoglobin 17.9H, Deoxyhemoglobin [redacted] Carboxyhemoglobin [redacted] Methemoglobin [redacted] Blood Gas Vent Mode NASAL CANNULA, Blood Gas Inspired Oxygen 36, Blood Gas Liter Flow [redacted]

Coagulation

[redacted]

Prothrombin Time 11.6H, Prothrombin Time INR 1.1, Activated Partial Thromboplast Time [redacted] [redacted] D-Dimer Quantitative (PE/DVT) 12.05H

Toxicology

[redacted] Ethyl Alcohol Level 12H

Urine Opiates Screen NEGATIVE, Urine Methadone Screen NEGATIVE, Urine Fentanyl Screen NEGATIVE, Urine Barbiturates Screen NEGATIVE, Urine Phencyclidine Screen NEGATIVE, Urine Amphetamines Screen NEGATIVE, MDMA (Ecstasy) Screen NEGATIVE, Urine Benzodiazepines Screen NEGATIVE, Urine Cocaine Screen NEGATIVE, Urine Cannabinoids Screen PRESUMPTIVE POSITIVE, Urine Drug Screen Comment

Serology

[redacted]

Influenza Type A Antigen Presumptive Negative, Influenza Type B Antigen Presumptive Negative, Respiratory Syncytial Virus Antigen Not Detected, SARS-CoV-2 Antigen (Rapid) NEGATIVE

[redacted]

Urines

Test	[redacted]
Urine Color	YELLOW (YELLOW)
Urine Clarity	CLEAR (CLEAR)
Urine pH	6.5 (4.5-8.0)
Urine Specific Gravity	1.015 (1.005-1.030)
Urine Protein	3+ (NEGATIVE) H
Urine Glucose (UA)	TRACE (NEGATIVE) H
Urine Ketones	NEGATIVE (NEGATIVE)
Urine Occult Blood	1+ (NEGATIVE) H
Urine Nitrite	NEGATIVE (NEGATIVE)
Urine Urobilinogen	[redacted] E.U./dL (0.2-1.0)
Urine Leukocyte Esterase	NEGATIVE (NEGATIVE)
Urine RBC	[redacted] /hpf [redacted] H
Urine WBC	[redacted] /hpf (0-2) H
Urine Bacteria	TRACE (NONE) H

Radiology

Imaging

ONE VIEW AP PORTABLE CHEST X-RAY.

INDICATIONS: Sepsis [redacted]

COMPARISON: None available

FINDINGS: There has been prior sternotomy. There has been aortic valve replacement surgery. The ribs and clavicles are intact and there is cardiomegaly. There is bilateral interstitial infiltrative pattern right greater than left suggesting congestive heart failure and interstitial pulmonary edema. No evidence of pneumothorax or pleural effusion.

IMPRESSION:
1. Prior sternotomy with aortic valve replacement surgery.
2. Cardiomegaly with congestive heart failure and interstitial pulmonary edema right greater than left

---

Dictated By [redacted]
Dictated Date/Time [redacted]

Electronically Signed By [redacted]
Signed Date/Time [redacted]

Impression

Dx
Acute respiratory failure with hypoxia
Heart failure acute on chronic with systolic dysfunction
Paroxysmal atrial fibrillation
History of asthma
Hypertension
History of valvular heart disease, status post aortic valve bioprosthelic valve replacement

Homeless Screening/Assessment

Do PT reside in temp living: No
Do PT has nighttime shelter?: No
Do PT have a fixed residence?: Yes

Assessment

Comments
This is a 55-year-old male with a history of heart disease that being congestive heart failure, status post aortic valve replacement with bioprosthetic, atrial fibrillation and hypertension who presented to the emergency room with severe shortness of breath. He denies a productive cough. He denies fever.
In the emergency room the patient required a BiPAP for adequate oxygenation. An arterial blood gas showed a pH of 7.16. He was in need of inpatient care and these arrangements were made.
The patient had an echocard[redacted]e beginning of this year showing an LVEF of [redacted]
The patient has been admitt[redacted] Pulmonary and cardiology consultation has been called
Continue to diurese. Continue to wean as able off BiPAP
Social services assistance as patient is homeless

Plan

Plan
Admit t[redacted]
See orders
Follow clinically

Other

Note
Considering this patient's current medical condition and co-morbidities, we anticipate at least 2 overnight stays. Further treatment will be based upon the hospital course and response to treatment.

General Conditions [redacted]
Other justification for a [redacted]
Respiratory failure, CHF [redacted]

[redacted]

Electronically Signed by [redacted]
Electronically Co-Signed by:

[redacted]

CARDIOLOGY REPORT
[redacted]

[redacted]

Exam: Echo 2D w/ SPEC/Color Complete
Service Date & Time:
Ordering Dr [redacted]

Location [redacted]
Order Date & Time [redacted]
Attending Dr [redacted]

[redacted]

[redacted]

Date of Study [redacted]

---------- APPROVED REPORT ----------

EXAM: Comprehensive 2D, Doppler, and color-flow Echocardiogram
DOS: [redacted]

Patient Location: [redacted]

BSA: 2.17

HR: 57 bpm

Indications: SOB, CP

Left Ventricle
The left ventricle is normal size. Left ventricular systolic function is severely [redacted] Moderate concentric left ventricular hypertrophy. Moderate diastolic dysfunction is present (pseudonormal filling). No left ventricle thrombus noted on this study. LVEF is [redacted]

Right Ventricle
The right ventricle is normal size. Right ventricular systolic function is mildly reduced.

Atria
Left atrium is mildly dilated. Right atrium is mildly dilated. The interatrial [redacted] is intact with no evidence for an atrial [redacted] defect.

Aortic Valve
Prosthetic aortic valve is normal in appearance. Prosthetic aortic valve is possibly stenotic. AVA=1.1 cm2 Mild aortic regurgitation.

[redacted]

[redacted]

CARDIOLOGY REPORT

[redacted]

[redacted]

Exam: Echo 2D w/ SPEC/Color Complete
Service Date & Time:
Ordering Dr [redacted]

[redacted]

Location [redacted]
Order Date & Time: [redacted]
Attending Dr [redacted]

Mitral Valve
The mitral valve is normal in structure. Normal mitral valve excursion. Mild to moderate mitral regurgitation. There is no evidence of mitral valve vegetations.

Tricuspid Valve
The tricuspid valve is normal in structure. Mild tricuspid regurgitation.

Pulmonic Valve
Pulmonic valve is not well visualized. There is no pulmonic valvular regurgitation.

Great Vessels
The aortic root is normal in size. The ascending aorta is normal in size. IVC is normal in size and collapses >50% with inspiration.

Pericardium
There is no pericardial effusion.

2D Dimensions

IVSd	1.7 cm M: [redacted]	LVEF (Visual)	[redacted]
%			
PWd	1.7 cm M: 0.6 - [redacted]	LVEF (Simpson's)	21.5
% M: 52 - 72			
LVDd	[redacted] cm M: [redacted] - 5.9	LV Volume	158.1
mL M: 62 - 150			
LVDs	[redacted] cm M: 2.5 - [redacted]	LV Volume Index	
[redacted] mL/m2 M: 34 - 74			
Left Atrium	[redacted] cm M: [redacted]	LA Volume	[redacted] mL
LVOT	[redacted] cm (M/F) [redacted]	LA Volume Index	
43.7 mL/m2 (M/F) [redacted]			
Ascending Aorta	3.0 cm M: [redacted]	IVC	[redacted]
cm			
TAPSE	1.7 cm (M/F) >1.7		

M-Mode Dimensions
MV EPSS 1.9 (<0.5cm) TAPSE 1.7 (>1.7)

LV Diastology

[redacted]

[redacted]

CARDIOLOGY REPORT
[redacted]

[redacted]

[redacted]

Exam: Echo 2D w/ SPEC/Color Complete
Service Date & Time:
Ordering Dr [redacted]

Location [redacted]
Order Date & Time [redacted]
Attending Dr [redacted]

E/A Ratio 3.7
LAT E' [redacted] m/sec)
MED E' 0.05 (< [redacted] m/sec)
E/E' Average [redacted]

Aortic Valve
LVOT Max 0.85 m/s
LVOT VTI [redacted] cm
AoV Peak Vel. 2.47 m/s
AI PHT 0.68 m/sec
AO Peak GR. [redacted] mmHg
AO Mean GR. [redacted] mmHg
AVA (VTI) 1.1 cm2

Mitral Valve
MV E Max [redacted] m/s
MV A Velocity [redacted] m/s
MV Decel. Time 177.6 ms
MV Mean Gr. [redacted] mmHg
MR Vmax [redacted] m/s
MR Max PG 105.67 mmHg

Pulmonary Valve
PV Peak Velocity [redacted] m/s)
PV Max PG [redacted] mmHg

Tricuspid Valve
TR P. Velocity [redacted] m/s
RVSP [redacted] mmHg
TR Max PG [redacted]
RV S' [redacted] m/s

Conclusion
Left Ventricle : Left ventricular systolic function is severely [redacted] LVEF is [redacted] Moderate diastolic dysfunction is present (pseudonormal filling).

Physician Signed [redacted]

Electronically Signed by:
[redacted]

[redacted]

[redacted]

[redacted]

Note started by: [redacted] Note started date/time: [redacted]

Cardiac Cath Procedure [redacted]

Date & Time

Date of Service

[redacted]

Time of Evaluation
10:01

Procedure

Notes:

Cardiac Catheterization Report (Radial Approach)

Date of Procedure: [redacted]

Attending Physician [redacted]

Referring Physician: [redacted]

INDICATIONS:
NSTEMI
Acute systolic chf

OTHER PERTINENT MEDICAL HISTORY:
history of AVR

Procedure class: urgent, inpatient
CCS: 3
NYHA: 4

PROCEDURES PERFORMED:
Ultrasound guided retrograde right radial arterial access.
Prophlyactic injection of verapamil and lidocaine through radial artery for vasospasm prevention
Selective left and right coronary angiography.
Moderate sedation 30 minutes

MEDICATIONS GIVEN:
Heparin 4000 mg IV
Midazolam, benadryl and fentanyl IV for sedation

DESCRIPTION OF PROCEDURE
The risks, benefits and objectives of the procedure were discussed with the patient. The risks associated with cardiac catheterization +/- PCI including bleeding, MI, arrhythmia, stroke, kidney damage and death were explained to patient. They indicated understanding and provided both verbal and written informed consent to proceed. The patient was brought into the cardiac catheterization lab and prepped in the usual sterile fashion.

Conscious sedation was achieved with iv midazolam, iv benadryl and iv fentanyl. Local anesthesia was achieved with 1% Lidocaine. Utilizing ultrasound guidance and modified seldinger technique, retrograde right radial arterial access was gained with placement of a [redacted] slender French sheath. Vasospasm

[redacted]

prophylaxis was achieved with intra arterial lidocaine and verapamil. Selective right and left coronary angiography was performed with a 5-French Tiger catheter. Left heart catheterization, was not performe[redacted] due to presence of prosthetic aortic valve. All catheters and wires were remove and hemostasis was achieved with a TR band device.

I[redacted] was present for and participated in the entire case.

INTRAPROCEDURAL COMPLICATIONS: None.

FLUORO TIME: 4 minutes

AIR KERMA: see accompanying cath lab report

TOTAL CONTRAST USED: 25 mls of visipaque contrast

FINDINGS:

HEMODYNAMICS
1. Heart rate was 65 bpm in atrial flutter
2. Aortic Pressure was 114/45 mmHg

SELECTIVE CORONARY ANGIOGRAPHY
1. The left main was a large vessel which gave rise to the LAD and circumflex. There was no significant stenosis.
2. The LAD was a large vessel which wrapped around the apex. There was mid to distal LAD luminal irregularities with no significant stenosis.
3. The Left circumflex was a large vessel that became small as it traveled through the av groove. There was no significant stenosis.
4. The RCA was a large, dominant vessel which distally bifurcated into PLB and PDA branches. There was no significant stenosis.

CONCLUSIONS:
1. Mild non obstructive CAD

RECOMMENDATIONS
1. Aggressive risk factor modification.
2. Routine post access site management

[redacted]
Interventional Cardiology

[redacted] [redacted]

Electronically Signed by [redacted]
Electronically Co-Signed by:

[redacted]

[redacted]

Signed

[redacted]

[redacted]

[redacted]

Exam: RAD/CHEST X-RAY 1V
Ord Physician [redacted] **Locatio** [redacted] **Status:** ADM IN

Chest one view at 0539

Indication: Follow. CHF

Comparison [redacted]

Findings: The regional skeleton is unremarkable. Heart remains enlarged. There is previous cardiac surgery. There is improvement of the pulmonary edema. No infiltrates are seen. The pleural surfaces are normal.

Impression:

Cardiomegaly

Previous cardiac surgery

Improving pulmonary edema

Dictated By [redacted]
Dictated Date/Time [redacted]

Electronically Signed By [redacted]
Signed Date/Time [redacted]

[redacted]

[redacted]

[redacted]

[redacted]

Date: [redacted] Test Time: [redacted]
Pat Name: [redacted] Department:
Patient ID: [redacted] Room:
Gender: M Technician: [redacted]
DOB: [redacted] Requested By:
Order Number: [redacted] Reading MD: [redacted]

Measurements

Intervals		Axis	
Rate:	56	P:	0
PR:	0	QRS:	-103
QRSD:	121	T:	239
QT:	676		
QTc:	653		

Interpretive Statements

Atrial flutter with predominant 4:1 AV block
RBBB and LAFB
Probable lateral infarct, age indeterminate
Abnormal T, probable ischemia, widespread
Borderline ST elevation, anterior leads
Prolonged QT interval
When compared with traci[redacted] now diffuse T wave inversion.
Electronically Signed On[redacted]

Please click the below link to view image of tracing.

Physician Signed: [redacted]

Electronically Signed by:

[redacted]

[redacted]

[redacted]

[redacted]

Age/Sex: 55/M

[redacted]

Type: Surgery Phase 1 Notes

[redacted]:
0945 RECEIVED FROM CATH LAB VIA BED ACCPD BY [redacted] AWAKE BUT APPEAR SLEEPY. BREATHING EASY ON ROOM AIR. [redacted] PLACED BACK ON 2L/NC. AFLUTTER ON THE MONITOR WITH CONTROLLED [redacted] BP INITIALLY >150SYSTOLIC. HANDS COLD.FEELING COLD EARLIER. BLANKETS REPLACED WITH WARMER BLANKETS. DENIES PAIN.DENIES SOB. RT RADIAL SITE WITH TRBAND AND WICK DRESSING WITH SMALL BLOOD STAIN. INSTRUCTED TO KEEP RT WRIST STRAIGHT AND AVOID HEAVY LIFTING AND PUSHING.VERBALIZED UNDERSTANDING. HOB ELEVATED. RT HAND ELVATED.

1000 ASLEEP. BP IMPROVING GRADUALLY. REMAINED AFLUTTER W/ HR [redacted]

1045 EASILY AROUSABLE. TRBAND CARE STARTED.NO BLEEDING OR SWELLING NOTED.OFFERED PO LIQUIDS AND TOLERATED WELL. AFLUTTER WITH HR <40/MIN TO 50/MIN. ASLEEP. NO COMPLAINTS GIVEN. [redacted] ON ROOM AIR.

[redacted]:
1100 TRBAND CARE IN PROGRESS. NO BLEEDING OR SWELLING FROM RT RADIAL SITE.INTERMITTENTLY ASLEEP. AFLUTTER W/ HR=38-50/MIN. ASYMPTOMATIC.OFFERED URINAL TO USE FOR URINATION BUT REFUSED.

1115 TRBAND CARE COMPLETED. BILAT HANDS WARM TO TOUCH.W/ PALPABLE RT RADIAL PULSE,GOOD COLOR AND INTACT SENSATION.INSTRUCTED SIGNS OF BLEEDING,INTERNAL AND EXTERNAL ,TO WATCH FOR. INSTRUCTED TO CONTINUE TO REST RT HAND AND REST UNTIL 6PM TONIGHT AND AVOID BP TAKING AS WELL AS BLOOD DRAW FROM RT ARM FOR 24 HOURS. VERBALIZED UNDERSTANDING. PRESSURE DRESSING APPLIED TO RT WRIST. CONTINUED TO MONITOR FOR BLEEDING AND SWELLING. REPORT GIVEN TO FLOOR RN.

1200 VSS.BP<150SYSTOLIC. AFLUTTER W/ HR=62 [redacted] ON ROOM AIR.NO BLEEDING OR SWELLING FROM RT RADIAL SITE.

1215 TRANSFERRED BACK TO PT'S ROOM STABLE CONDITION.SENT W/ PATIENT TUNA SANDWICH THAT HE REQUESTED. ENDORSE [redacted] AT BEDSIDE.